Activity 3: Evidence and Technology Selection Board

Learning outcome: LO1 Objective: Identify and appraise wellbeing trends, technologies and research for workplace relevance Expected output: Source appraisal ledger, weighted option board and pilot hypothesis Primary source: https://www.ncbi.nlm.nih.gov/books/NBK201567/

Goal

Compare wearables, telehealth, wellbeing apps, gamification and AI coaching against evidence, need, safety, inclusion and implementation fit.

Real-world scenario

The NCBI review describes employee wellness programmes as employer-sponsored efforts to support healthy behaviour and reduce health risk, and stresses leadership, alignment, scope, accessibility, partnerships and communication. It also notes participation barriers such as awareness, time, manager support, perceived benefit, access and privacy.

BrightForge Engineering, a fictional manufacturer, has S$80,000 for one pilot. Vendors propose a wearable challenge, telehealth access, a mindfulness app with gamification, or AI coaching. The workforce includes production shifts, engineers and mature technicians.

Evidence boundary

Public case facts and fictional learner data must never be mixed. Every numerical or organisational training input is labelled as an assumption.

Training assumptions / guardrails

- Vendor claims, prices and implementation times are fictional and have not been independently validated.
- The workforce need is access to timely support and recovery, not increased monitoring.
- Mobile access and English literacy vary across groups.
- The board may reject all four options or select a non-technology response.

Questions

1. Which source claims are credible, relevant and transferable to this workforce?
2. What employee need does each option address-and what new risk does it create?
3. Which criteria and weights should govern the choice?
4. What pilot result would justify scale, adaptation or termination?

Detailed activity procedure

Step 1: Frame a PICO-style question

Define population, intervention, comparator and outcome for the workplace decision. State acceptable implementation time and risk boundaries.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 2: Build a source ledger

Record source type, date, population, intervention, outcome, limitations and conflicts of interest. Separate research from vendor assertion.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 3: Appraise transferability

Compare research context with workforce, setting, access, culture and baseline need. Rate direct, partial or weak relevance with reasons.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 4: Map option logic

For each technology, show need, mechanism, expected behaviour or condition change, outcome and possible harm.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 5: Choose decision criteria

Weight need-fit, evidence quality, privacy, safety, inclusion, usability, vendor capability, cost and evaluability before scoring options.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 6: Run the option board

Score each option independently, compare rationales, resolve material disagreements and record sensitivity to changed weights.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 7: Define the pilot

Write a testable hypothesis, target group, comparison or baseline, safeguards, reach and outcome measures, duration and stop triggers.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 8: Issue a decision note

Recommend one option, a combined/non-technology alternative or no pilot. State evidence gaps and due-diligence actions.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Deliverable

Source appraisal ledger, weighted option board and pilot hypothesis

Acceptance criteria

The recommendation distinguishes research from vendor claims, maps each option to a defined need, includes privacy and accessibility gates, tests weight sensitivity and states scale/adapt/stop criteria.

Reflection and debrief questions

1. Which assumption changed your recommendation most, and why?
2. What evidence would you request before relying on the result?

3. Which stakeholder owns the next decision or mitigation?
4. Which review trigger or future exercise should follow?

Source note

Source accessed 16 August 2026: https://www.ncbi.nlm.nih.gov/books/NBK201567/